



**State of Vermont
Office of the Secretary of State**

Office of Professional Regulation
89 Main Street, 3rd Floor
Montpelier, VT 05620-3402
sos.vermont.gov

**James C. Condos, Secretary of State
Christopher D. Winters, Deputy Secretary
S. Lauren Hibbert, Director**

August 24, 2021

Marc B. Heath, Esq.
199 Main Street
P.O. Box 190
Burlington, VT 05402

Case: In Re Gretchen Lewis
Credential: 068.0000569
Docket No. 2020-39, 2020-40, 2020-41, 2021-38

Dear Attorney Heath:

Enclosed please find a copy of the approved *Stipulation and Consent Order*

For questions about complying with this Order, please contact the Case Manager at 802-828-2875

Sincerely,

E-SIGNED by S. Lauren Hibbert
on 2021-08-24 12:14:13 EDT

S. Lauren Hibbert
Director

Enc: *Stipulation and Consent Order*

cc: Stephen Reynes, Esq., Hearing Officer
Jennifer Colin, Esq., Prosecuting Attorney
Carla Preston, Case Manager
Marc Heath, Esq., Attorney for Respondent, First Class Mail & Email

**STATE OF VERMONT
SECRETARY OF STATE
OFFICE OF PROFESSIONAL REGULATION
BOARD OF ALLIED MENTAL HEALTH PRACTITIONERS**

IN RE:	)	Docket No.	2020-39
Gretchen E. Lewis	)		2020-40
License No. 068.0000569	)		2020-41
	)		2021-38

STIPULATION AND CONSENT ORDER

STIPULATION

NOW COME the State of Vermont, by and through State Prosecuting Attorney Jennifer B. Colin, and Respondent Gretchen E. Lewis, by and through her counsel, Marc Heath, Esq., who hereby Stipulate and Agree as follows:

Board Authority

1. The Vermont Board of Allied Mental Health Practitioners (the "Board") has jurisdiction to investigate and adjudicate allegations of unprofessional conduct committed by Licensed Clinical Mental Health Counselors ("LCMHC"); issue warnings or reprimands, suspend, revoke, limit, condition, deny, or prevent the renewal of licenses if, after disciplinary hearing, it finds that Respondent has engaged in unprofessional conduct. 3 V.S.A. §§129, 129a; 26 V.S.A. Chapter 65; and the Administrative Rules for the Board of Allied Mental Health Practitioners, and the Rules of the Vermont Office of Professional Regulation.

Stipulated Facts

2. Gretchen E. Lewis ("Respondent") of Derby Line, Vermont is licensed by the State of Vermont as a Clinical Mental Health Counselor under license number 068.0000569. The license was first issued on November 15, 2001 and expires on January 31, 2023.
3. Respondent is also licensed by the State of Vermont as an Alcohol and Drug Counselor ("LADC") under license number 151.0127939. This license was first issued on January 31, 2017 and expires on January 31, 2023.

Dual Relationship

4. In April 2019, OPR Investigators investigated a complaint against Respondent for a potential dual relationship for providing counseling services as a LCMHC to her adult niece.

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5. In communications with OPR Investigators, Respondent stated she did not provide direct counseling to her niece, but that her niece was in crisis after being locked out of her home by her husband, had two small children, and needed various referrals for resources.
6. On December 19, 2018, Respondent, as an LADC and LCMHC for Lewis Counseling Services, LLC, completed and signed an Intake Form, which identified her niece as the "patient" and Medicaid as the patient's Primary Insurance Carrier.
7. Respondent signed the Intake Form for the patient, stating "Signature on file."
8. On three occasions between December 20, 2018 and February 28, 2019, Respondent completed notes for counseling sessions with her niece and signed the notes with her credentials LCMHC/LADC.
9. On April 5, 2019, Respondent wrote a letter in support of her niece to have a companion animal in her housing, providing a clinical diagnosis of depression and signing the letter with her credentials as LCMHC and LADC.

"Provisional Diagnosis" of a Non-Client

10. According to Respondent, Respondent provided counseling to J.B. ("Client 1") five or six years ago but has only seen him twice since 2014.
11. In 2019, Respondent met once with Client 1, at which time Client 1 expressed that he and his girlfriend, whom Respondent did not know, were having financial difficulties.
12. Respondent provided Client 1 with a form that Respondent partially completed for Client 1's girlfriend to obtain general financial assistance and benefits from the State of Vermont.
13. The partially completed form that Respondent provided to Client 1 for his girlfriend had a "provisional diagnosis" of Client 1's girlfriend of Post-Traumatic Stress Disorder ("PTSD").
14. Respondent never met with Client 1's girlfriend, nor did Client 1's girlfriend seek out assistance from Respondent.
15. Respondent did not obtain informed consent from Client 1's girlfriend to render professional services or to make a provisional diagnosis for the purpose of obtaining state benefits.
16. Respondent's notes for her session with Client 1 in 2019 also noted that Respondent provided the partially completed form for Client 1's girlfriend with a provisional diagnosis of PTSD and that Client 1's girlfriend needed to attend "their next appointment."

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Boundary Issues

17. For approximately 20 years, Respondent has been the mental health counselor for an adult female client ("Client 3") who is disabled.
18. Client 3 is in her 60's and has various medical issues, including a serious condition that results in Client 3 requiring frequent medical care and/or hospitalizations.
19. Though Client 3 has medical, cognitive and mental health issues, she lives independently and makes her own decisions.
20. Respondent and Client 3 entered a therapist-client relationship when Respondent worked at a Facility in the early 2000's. The counseling relationship continued when Respondent went into private practice in 2005.
21. Respondent generally has sessions with Client 3 twice each week.
22. Respondent does not generally see Client 3 at her office. Instead, Respondent picks Client 3 up in Respondent's vehicle at Client 3's apartment and they drive around, go to Dunkin Donuts or McDonald's for coffee or a soda, go to the pharmacy to pick up Client 3's medications, and do similar errands.
23. Sometimes Respondent has sessions with Client 3 in Client 3's apartment.
24. Respondent bills Medicaid for "psychotherapy" services in one-hour increments for counseling sessions, including times when she picks up Client 3 and is out in the community for errands or to get a beverage.
25. When Client 3's father was in a nursing home prior to his death, Respondent would drive Client 3 to the nursing home to visit with her father. Respondent would be present with Client 3 during the visit with her father and then use the time during the car ride back to Client 3's apartment to process the interaction between Client 3 and her father.
26. In addition to seeing Client 3 twice during the week, Respondent meets Client 3 at a local restaurant for coffee sometimes on Saturdays.
27. Respondent told OPR's investigator that she does not generally bill Client 3 for a session when they meet at the restaurant because the meetings are not private.
28. However, in July 2020, Respondent did make a session note and bill for "psychotherapy" for one Saturday when she met with Client 3 at a restaurant, stating: "Met Client at her favorite breakfast restaurant today so that she could introduce all her acquaintances there;" and "Interpersonal Therapy focuses on client being able to connect with people who are important to her and introduce them to one another."

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29. Respondent does not have written releases for meetings with Client 3 in the community or in the presence of others, nor do Client 3's notes reflect any counseling with Client 3 regarding consequences of boundary extensions or precautions Respondent took regarding boundary extensions.
30. Client 3 has the benefit of a special needs trust that provides for her financial needs, and her brother assists her with the trust.
31. Respondent is aware of Client 3's special needs trust and has, over the years, written letters in support of Client 3 accessing the trust for certain expenses.
32. In June 2013, Client 3 executed a Durable Power of Attorney naming her brother as her agent with the authority to act in her name and stead with respect to all of Client 3's property, including accounts, benefits, real property, and other financial interests, irrespective of Client 3's capacity or ability to act on her own behalf.
33. Client 3 named Respondent as her alternate agent in the Durable Power of Attorney, with Respondent having the authority to act in Client 3's name and stead with respect to all property owned by Client 3, including bank accounts, financial benefits, real property, and other financial interests, in the event that Client 3's brother was unwilling or unable to serve as Client 3's agent.
34. On June 21, 2013, Respondent signed Page 5 of Client 3's Durable Power of Attorney, entitled "Acknowledgement of Agent" in which Respondent verified by her signature that she accepted the appointment as Client 3's agent and understood the duties of the power of attorney under the law.
35. Respondent remained as Client 3's Alternate Agent for her Durable Power of Attorney until the investigation of this case in 2020, though she informed OPR's investigator that she has never taken any action or exercised the powers granted to her as Alternate Agent.
36. During a counseling session, Client 3 informed Respondent that an individual who worked as a clerk at a local convenience store and who had also been employed by Client 3 to do Client 3's grocery shopping ("Individual") was communicating information to Client 3 that Client 3 found upsetting.
37. During a counseling session, Respondent accompanied Client 3 to the local convenience store where the Individual worked for the purpose of Respondent speaking to the Individual and directing the Individual to cease communicating the particular information to Client 3 that Client 3 found upsetting.
38. Prior to going into the convenience store, Respondent's interaction with Client 3 to obtain consent was that Respondent informed Client 3 that Respondent's intention in taking Client 3 into the store was to speak with the Individual and tell the Individual to stop communicating the information to Client 3, to which Client 3 responded "ok" and words to the effect of "let's go."

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39. Respondent did not seek a written release from Client 3.
40. Respondent indicated to OPR's investigator that she did not identify herself to the Individual as Client 3's therapist upon initiating contact in the store, as the Individual already knew Respondent was Client 3's therapist because Respondent had spoken with the Individual by phone previously in the presence of Client 3 and Respondent presumed that the Individual was aware that she was Client 3's therapist because people in the community know who she [Respondent] is.
41. Respondent stated there were other people in the convenience store when she accompanied Client 3 in and spoke with the Individual who was working there, but that Respondent made sure to take the Individual aside to make the conversation as private as possible.
42. Respondent has no written releases or progress notes regarding the interaction with the Individual in public in the presence of Client 3 or any counseling with Client 3 prior to the interaction documenting the rationale for the interaction or the anticipated consequence for the client and for others involved.
43. In her interview with OPR investigator, Respondent stated that she sometimes pays for client's prescriptions when she picks them up from the pharmacy, but only if the client can reimburse her with cash at the time.

Violation One: 3 VSA 129a(b)(2) Failure to conform to essential standards of acceptable and prevailing practice.

44. Paragraphs 2 through 43 above are incorporated herein.
45. The Administrative Rules of the Board of Allied Mental Health Practitioners ("ARBAMHP") provide that the ethical codes of the National Board for Certified Counselors ("NBCC") and the American Counseling Association ("ACA") establish the standard of practice and fitness within the Allied Mental Health professions. *See* ARBAMHP Rule 6.6.
46. Essential standards of acceptable and prevailing practice for Licensed Clinical Mental Health Counselors in Vermont prohibit counselors from engaging in counseling relationships with friends or family members with whom they have an inability to remain objective. *See* ACA Code of Ethics, Standard A.5.d.
47. Paragraphs 2 through 9 above demonstrate that Respondent engaged in a counseling relationship with a family member with whom Respondent had the inability to remain objective.
48. Essential standards of acceptable and prevailing practice for Licensed Clinical Mental Health Counselors in Vermont require that the LCMHC obtain informed consent from a client to enter a counseling relationship. *See* ACA Code of Ethics, Standard A.2.a.

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49. Paragraphs 10 through 16 demonstrate that Respondent failed to obtain informed consent from Client 1's girlfriend prior to partially completing a form for state benefits, provisionally diagnosing Client 1's girlfriend with PTSD, and revealing the provisional diagnosis to Client 1 without consent.
50. Essential standards of acceptable and prevailing practice for Licensed Clinical Mental Health Counselors in Vermont require counselors to provide accurate reports based on direct experiences with individual(s) or documentation from other professionals when providing opinions. *See* NBCC Code of Ethics, Standard 43.
51. Paragraphs 10 through 16 demonstrate that Respondent failed to conform to essential standards of acceptable and prevailing practice when she provided a provisional diagnosis of Client 1's girlfriend without having direct experience with her and without having documentation from other professionals.
52. The essential standards of acceptable and prevailing practice require a Licensed Clinical Mental Health Counselor to consider the risks and benefits of extending counseling relationships beyond the conventional parameters and to take appropriate professional precautions such as informed consent, consultation, supervision, and documentation to ensure that judgment is not impaired and no harm occurs. *See* ACA Code of Ethics, Standard A.6.b.
53. Paragraphs 22 through 48 demonstrate that Respondent failed to conform to the essential standards of acceptable and prevailing practice by acting in the following manner without considering the risks and benefits and without taking appropriate professional precautions such as informed consent, supervision and documentation to ensure that no harm occurred: engaging with Client 3 in public; having therapy sessions in unconventional settings; attending Client 3's visits with her declining father; engaging in conversation with a third party in public in the presence of Client 3 about a situation that Client 3 revealed, in counseling, that she found upsetting; and paying for client medications and having them reimburse with cash.
54. The essential standards of acceptable and prevailing practice require a Licensed Clinical Mental Health Counselor to officially document boundary extensions, including the rationale for the interaction, the potential benefit, and anticipated consequences of the boundary extension.
55. Paragraphs 17 through 43 demonstrate that Respondent failed to conform to the essential standards of acceptable and prevailing practice by engaging in boundary extensions with clients and failing to document those extensions, including the rationale, the potential benefits and the anticipated consequences for clients and others.
56. The acts, omissions, and/or circumstances described above constitute grounds for discipline because Respondent has committed unprofessional in violation of 3 V.S.A. §129a(b)(2).

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Violation Two: 3 V.S.A. §129a(a)(15) Failing to exercise independent professional judgment in the performance of licensed activities when that judgment is necessary to avoid action repugnant to the obligations of the profession.

57. Paragraphs 2 through 43 above are incorporated herein.
58. In Vermont, a Licensed Clinical Mental Health Counselor must exercise independent professional judgment in the performance of licensed activities when that judgment is necessary to avoid action repugnant to the obligations of the profession.
59. Paragraphs 2 through 16 above demonstrate that Respondent failed to exercise independent professional judgment when she provided counseling to her niece and failed to obtain informed consent before provisionally diagnosing a non-client with a mental health condition without having direct contact with the non-client.
60. Paragraphs 17 through 43 demonstrate that Respondent failed to exercise independent professional judgment in the performance of licensed activities when that judgment was necessary to avoid action repugnant to the obligations of the profession, including: engaging with Client 3 in public without proper consent or counseling; having therapy sessions in unconventional settings without proper consent or counseling; attending Client 3's visits with her declining father; engaging in conversation with a third party in public in the presence of Client 3 about a situation that Client 3 revealed, in counseling, that Client 3 found upsetting without proper consent or counseling; and paying for client medications and having clients reimburse Respondent with cash.
61. The acts, omissions, and/or circumstances described above constitute grounds for discipline because Respondent has committed unprofessional conduct in violation of 3 V.S.A §129a(a)(15).

Understandings

62. Respondent admits that the facts above are true and that the conditions below are necessary to protect the public.
63. Respondent understands that the Board must review and accept the terms of the Order set forth below and that if the Board rejects all or any portion of the Order, then this entire document shall be null and void.
64. Respondent specifically waives any claim that any disclosures made to the Board during the Board's review of this agreement prejudice Respondent's right to a fair and impartial hearing if this agreement is not accepted by the Board and proceeds to a hearing on the merits before the same Board.
65. Respondent has read and reviewed this document fully and agrees that it contains the entire agreement between the parties.

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66. This Stipulation and Consent Order is entered into voluntarily by Respondent after the opportunity to consult with legal counsel. Respondent has not been coerced by anyone into signing this Stipulation and Consent Order.
67. Respondent voluntarily waives the right to a contested hearing before the Board and waives any right to appeal from this Stipulation and Consent Order.
68. Respondent is of sound mind and is not under the influence of drugs or alcohol when signing this Stipulation and Consent Order.
69. Respondent agrees that the Order set forth below may be entered by the Board.
70. Respondent understands that she is responsible for all compliance costs associated with this Stipulation and Consent Order.

ORDER

Based upon the above stipulation, it is **ORDERED AND ADJUDGED** as follows:

- A. Respondent's Licensed Clinical Mental Health Counselor license is hereby **REPRIMANDED**.
- B. Respondent's Licensed Clinical Mental Health Counselor license is hereby **CONDITIONED** commencing with the date of entry of this Stipulation and Consent Order. The conditions are as follows:
 1. Issuance of License: Upon the commencement of these conditions, Respondent shall be issued a license labeled "conditioned."
 2. Length of Time Conditions Imposed: The conditions shall remain in place until Respondent has completed all conditions as ordered. Respondent shall be subject to the conditions until Respondent completes a **minimum of two (2) years** of practice in which she works at **least eighty (80) hours per month** as a LCMHC or LADC (or a combination of the two). Part time hours of fewer than eighty (80) hours every month shall be credited on a prorated basis. Respondent shall be prohibited from working in any OPR licensed capacity (i.e. as a LCMH, an LADC or any combination of the two) for more than forty (40) hours per week during the conditioned period.
 3. Required Coursework: Respondent shall, within ninety (90) days following the entry of this Order, at her own expense, complete twelve (12) hours of pre-approved online courses, which must include education on ethics, boundaries, dual relationships, ethical considerations while practicing in a small community, and risk management. The coursework required by this condition may also be used to satisfy the same condition in the Stipulation and Consent Order in the reciprocal discipline against Respondent's Alcohol and Drug Counselor license. For pre-approval, Respondent must submit to the Case Manager the name of each course,

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the name of the sponsoring organization, and a description of the course content and/or syllabus. Respondent must achieve a passing score on the post-tests for the courses, and submit test results, workbooks completed in full and in a satisfactory manner, and certificates of completion to the Office of Professional Regulation.

Failure to complete the courses within the prescribed timeframe shall constitute a violation of this Order. The coursework may not be counted toward any continuing education requirement of licensure.

4. Notification to Employers/Program Director: Respondent shall provide a copy of this Stipulation and Consent Order to all employers in any current or future setting in which Respondent practices in any mental health field as an OPR licensed professional (LCMHC or LADC) and inform them of Respondent's conditional license status. Within ten (10) days of the date of entry of this Consent Order or of any subsequent mental health employment, Respondent shall cause Respondent's immediate supervisor to submit to the Case Manager for approval an employer agreement form, acknowledging receipt of the Stipulation and Consent Order and the ability to comply with the conditions in the Consent Order.
5. Reports from Employers/Program Director. Within one (1) month of the date of this Order or within one (1) month of commencing employment in a work setting other than her solo private practice and **monthly** thereafter, Respondent shall cause every employer Respondent has worked for during the month to submit an evaluation of Respondent's work performance and attendance during that month. These reports shall be submitted in writing on forms issued or approved by the Enforcement Case Manager and shall indicate satisfactory performance and attendance.
6. Supervision. Respondent must have general supervision of her practice by a pre-approved LCMHC or LADC licensed in Vermont or other pre-approved licensed mental health practitioner, who has practiced for more than ten years and who is in good standing with the State of Vermont.

In the event Respondent is not employed by an agency that can provide such supervision, Respondent shall, at her own expense and within one (1) month of the date of this Order, engage a pre-approved supervisor ("Supervisor"), who is a Vermont-licensed LCMHC, LADC, PhD Psychologist or other licensed and pre-approved mental health practitioner who has practiced for more than ten years, to provide supervision regarding profession-specific issues or concerns that arise in Respondent's day-to-day practice, bearing in mind rules and regulations regarding patient privacy. Supervision must include discussions about compliance with standards of practice, privacy laws, boundaries, documentation, treatment plans and the therapeutic rationale for various clinical decisions, such as benefits and risks of continuing treatment with a client long-term, frequency of sessions, meeting with a client outside of the office, self-disclosure, ethical boundaries and other ethical and clinical issues. Respondent's consultation with her Supervisor shall take place no less than once per week by videoconference or in person.

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The pre-approved Supervisor shall provide monthly reports identifying when each supervision meeting occurred, how long it lasted, and generally identifying the profession-specific issues discussed. The Supervisor shall also report any ethical violations, boundary transgressions, or other practice issues or concerns. Respondent shall be responsible for locating a Supervisor and sending the name, contact information, credentials and curriculum vitae of the proposed Supervisor to the Enforcement Case Manager as part of the pre-approval process.

7. Out-of-State-Practice. Before any out-of-state practice can be credited toward fulfillment of these terms and conditions, Respondent shall first obtain approval prior to Respondent practicing as a LCMHC outside the State of Vermont. If Respondent fails to receive such approval before practicing as a LCMHC outside the State of Vermont, none of the time spent practicing out-of-state will be credited toward the fulfillment of the terms and conditions of this Consent Order. Approval will be given for out-of-state LCMHC practice if the Director of Professional Regulation, in his or her sole discretion, finds that Respondent's planned out-of-state practice does not compromise the supervisory or protective purposes of this Stipulation and Consent Order.
8. Notification of Place of Employment/Personal Address/Telephone Number. Within five (5) days of the date of entry of this Consent Order, Respondent shall notify the Office of Professional Regulation, in writing, of her current place of employment, personal address, and telephone number, as well as the name of any mental health educational programs in which she is enrolled. Respondent shall further notify the Office of Professional Regulation, in writing, within forty-eight (48) hours of any change in employment, personal address, or telephone number.
9. Notification to Other States. In the event Respondent is licensed in a mental health profession in any other state(s), she must inform the licensing Board of the state(s) in which Respondent is licensed of the conditional status of her Vermont LCMHC license within thirty (30) days of the date of entry of this Order. If Respondent fails to provide such notification, it will be considered a violation of this Order.
10. License Renewal. If Respondent's license expires while this Order is still in effect, this Order does not automatically extend the license. In that situation, in order to continue to practice as a LCMHC, Respondent must timely apply for renewal, pay the applicable fee and demonstrate that she has otherwise complied with the requirements for license renewal.
11. Costs. Respondent shall bear all costs of complying with this Consent Order.
12. Violation of this Order. Any violation of this Order may be a basis for additional disciplinary action. If a complaint of unprofessional conduct is made against Respondent during the term of this Order, this Order shall be automatically extended until the unprofessional conduct matter is concluded.

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13. Completion of Conditional License Period. Respondent shall submit a petition to the Docket Clerk to remove any and all conditions from Respondent's license. The State will assent to or oppose the petition. The Board will conduct a hearing if necessary. Respondent bears the burden of proof and must present evidence that Respondent has fully complied with the terms of this Order and these conditions are no longer necessary to protect the public.

C. Notwithstanding any provision above, Respondent must continue to meet all requirements to maintain a LCMHC license, for license renewal, and for license reinstatement.

D. This Stipulation and Consent Order is a matter of public record and may be reported to other licensing authorities as provided in 3 V.S.A. §129(a).

E. This Stipulation and Consent Order will remain part of Respondent's licensing file and may be used for purposes of determining sanctions in any future disciplinary matter.

AGREED TO:

Dated: August 18, 2021

STATE OF VERMONT
SECRETARY OF STATE
E-SIGNED by Jennifer Colin
on 2021-08-18 10:30:42 EDT
By: _____
Jennifer B. Colin
Prosecuting Attorney

Dated: _____

RESPONDENT
By: _____
Gretchen E. Lewis

APPROVED AS TO FORM:

Dated: August 18, 2021

RESPONDENT'S ATTORNEY
E-SIGNED by Marc Heath
on 2021-08-18 10:29:43 EDT
By: _____
Marc Heath, Esq.
Attorney for Respondent

APPROVED AND SO ORDERED:

Dated: August 24, 2021

VERMONT BOARD OF ALLIED
MENTAL HEALTH PRACTITIONERS
E-SIGNED by Lynn Irwin
on 2021-08-24 11:40:46 EDT
By: _____
Chairperson

Date of Entry: 8/24/21

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